

Introduction to the Still Technique

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Session Objectives

- Discuss the history of the Still technique
- Explain the principles of the Still technique
- Explain the physiologic mechanisms involved
- Discuss the indications and contraindications of the Still technique
- Explain how to apply the principles of Still Technique to treat any given somatic dysfunction based on your knowledge of anatomy and the function of the given region.

History of the Still Technique

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- Developed by:

Richard Van Buskirk, D.O., F.A.A.O

Dr. Still didn't teach techniques.

Based on writings of Charles Hazzard, D.O., and descriptions from Carl Phillip McConnell, D.O., and Herb Yates, D.O. who described the method as, "indirect and then direct."



Basic Principles of Still Technique

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- Passive
- Indirect
- Direct
- Muscle Spindle + Articulatory
 - Addresses the Muscular/Myofascial component AND the arthrodiol component of dysfunction

Basic Principles of the Still Technique

- Muscle Spindle Reflex
- Theory – Compressing muscles past their neutral position results in a “quieting” of gamma motor gain to the muscle spindle, leading to a resetting of the tension and hypertonicity of the muscle fibers.

Basic Principles of the Still Technique

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- Articulatory
- Brings dysfunction through its barrier

Principles of the Still Technique

(As described by Van Buskirk)

[Foundations of Osteopathic Medicine 5th ed. Ch 56 Still Technique: A Facilitated Indirect Then Direct Method](#)

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1. Evaluate the affected tissue and place it in its position of ease.
2. Introduce a force vector to the affected tissue from another part of the body. This compression or traction should be less than 2 lb. (1 kg).
3. Use the force vector to move the affected tissue, initially into a smooth path, to its position of ease. Then alter the position while maintaining this force toward and through to the area of restricted position and movement.
4. As the tissue moves through its restriction, a “bump” and/or a click may be felt or heard.
5. Passively move the region back to neutral and retest.

Indications

- Any condition where somatic dysfunction is involved

Contraindications of the Still Technique (1 of 3)

- Patient unable to tolerate treatment positions
 - Modify technique or use a different technique
- Radicular pain when performing to c-spine using compression (similar to Spurling Maneuver)
 - Reposition, try with traction, or try another technique

Contraindications of the Still Technique (2 of 3)

- Any condition of the region being treated where introduction of traction, or compression is contraindicated
 - Hip replacement
 - Shoulder pathology (certain treatment positions for Rib 1)
 - Chronic shoulder dislocation/separation (any treatment that moves and /or puts force into the shoulder)

Contraindications of the Still Technique (3 of 3)

- Recent trauma, fracture or dislocation at area involved with treatment
- Caution (but, not contraindication)
 - Osteoporosis
 - Malignancy
 - Rheumatologic Disorders
 - Congenital malformations
 - Stenosis
 - Other clinical diseases (use your clinical judgement)

Side Effects of the Still Technique

(similar to other osteopathic techniques)

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➤ Transient:

➤ Fatigue

➤ Soreness

➤ Stiffness

Due primarily to the restoration of circulation to previously dysfunctional tissue and release of muscular waste products.

Case

A 50-year-old female physician presents to your office complaining that she twisted her back wrong while treating a rather large patient with muscle energy treatment. She is having a constant sharp pain in the right lower back which is exacerbated with left rotation and deep breathing. Nothing makes it better.

Case (continued)

The pain is non-radiating. She has no associated numbness, tingling, weakness, or bowel or bladder function changes. She had an x-ray of her lumbosacral spine done yesterday and it was normal.

Case (continued)

PMHx: Raynaud's disease

PSx: 2 cesarean sections

Soc Hx: denies alcohol, drugs, tobacco use

Family History: both parents with HTN and 5 siblings alive and well

ROS: Only positive for the back pain

Case (continued)

Physical Exam:

Lower Extremity Motor 5/5 B/L

L4 and S1 reflexes 2/4 B/L

No gross sensory deficit in the lower extremities

Negative straight leg raise B/L

Negative costovertebral angle tenderness B/L

Case (continued)

Physical Exam (continued):

Negative FABER test B/L

B/L PVMS R>L

Right Psoas spasm

B/L hamstring spasm

L1 FRS right

Left piriformis hypertonic with associated CS point

Case (continued)

Assesment:

Low Back pain

Muscle spasm

Somatic Dysfunction – Lumbar

Somatic Dysfunction - Sacrum

Somatic Dysfunction – Lower Extremity

Case (continued)

Plan:

Low Back pain – Hamstring, psoas, Paraspinal mm. stretches, piriformis stretch

Muscle spasm – Stretches as above

Somatic Dysfunction – Lumbar

OMT – Soft tissue, Still Technique

Somatic Dysfunction – Sacrum

OMT – CS

Somatic Dysfunction – Lower Extremity

OMT - MET

Case (continued)

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How did we treat the L1 FRS right with Still
Technique?

Freedom's

Compress

Maintain compression and bring through barrier

Reassess

Summary

Still Technique:

- Passive, indirect, then direct technique
- Physiologic mechanism - Muscle spindle, articular
- Method – guide to freedoms, compress, maintain compression and guide into and through barrier, return to baseline, reassess

Core References

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DiGiovanna, Amen and Burns. An Osteopathic
Approach to Diagnosis and Treatment, 4th Ed

[Chapter 16: Still Technique](#)

Foundations of Osteopathic Medicine 5th ed.

[Ch 56 Still Technique: A Facilitated Indirect Then Direct
Method](#)

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